Three Voices: Uncertainty-Gated Rendering of a Maternal-Health Risk Alarm for Mothers, Community Health Workers, and Clinicians

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Abstract

Three Voices renders a single maternal-health risk prediction as three stakeholder-specific action-signals, one each for a clinician, a community health worker, and a mother, and gates each signal on the model's own uncertainty. Below a fixed top-two margin the highest-severity signal is withheld from the mother's channel and the health worker's, while the clinician's view of the split is untouched: the derate is red to amber, never red to green, and amber still says *needs follow-up*. Every string is a deterministic template filled from the case's SHAP attributions, with no language model at inference, so a rendering can be regenerated and audited line by line. Code, the rendered artifacts, the cross-validation and SHAP tables, and the evaluation matrix are archived, and all of it regenerates from five commands.

We also report a leakage audit of the UCI Maternal Health Risk benchmark. Of the 1012 rows that survive a two-row plausibility filter, 561 are exact duplicates of an earlier row, and retaining them moves macro-recall from 0.580 to 0.859 under the same cross-validation. That is a cost we quantify, not a fault we claim to have found first.

This is a design and feasibility artifact. Its evaluation is a formative single-evaluator heuristic critique of our own renderings, which found five defects, all of them corrected. That is an instrument that improved the design, not evidence that any tier helps a reader. No mother, health worker or clinician has used the system; the data is public, the task is generic maternal-health risk, and we make no clinical-validity claim. User studies with the three groups are the next step, gated on ethics clearance. The abstention idea sits between stakeholder-tailored explanation and selective prediction, and we claim only their pairing, not either mechanism.

Keywords: explainable AI; uncertainty; selective prediction; maternal health; community health workers; risk communication.

1 Introduction

A coin flip is not a red light.

A risk score computed at a rural antenatal visit is read by at least three people. A clinician wants the decomposition and can argue with it. A community health worker wants to know

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whether to refer this mother and what to write down. The mother wants to know what to do next, and she may not read at all, in which case the score reaches her as a colour and a spoken sentence or it does not reach her.

Most work on explaining a model to several audiences varies how much of the explanation each one gets. That leaves a second question open. When the model is not confident, which assertion should each channel carry?

Consider a case from the dataset we use. The model puts it at 0.013, 0.487 and 0.499 across low, mid and high risk. The top two are separated by 0.012, and the model is wrong: the record is true mid-risk. A clinician shown that split can see the call was nearly a tie. The mother, at the end of the same pipeline, has no way to discount anything, and if the rendering follows the argmax alone she is the one who receives an emergency generated by a coin flip.

Our response, which we call Three Voices, makes the abstention decision part of the rendering rather than part of the model. One band is computed per prediction from the class probabilities, and the highest-severity signal is raised only when the model is not on a knife edge. Below the threshold the mother's channel and the health worker's are both derated, while the clinician's is untouched. **The derate is red to amber, never red to green.** Amber still says *needs follow-up* and still sends her to a health worker. Nothing in this design removes a mother from care, and a reader who takes "not an emergency" to mean "not a concern" has read it backwards.

1.1 What we claim, and what we do not

We contribute three things, and they are different kinds of object: a rendering mechanism, the artifact that implements it, and a measurement on the benchmark underneath both.

A mechanism: abstention graded across stakeholder channels. The decision to withhold is taken once, from the model's output, then rendered as a different assertion for each reader rather than as the same assertion softened. The gate is a rule over class probabilities, stated with its grading in Section 4 and reproducible from the committed tables.

An artifact: an auditable rendering pipeline with no language model at inference. Every string a health worker or a mother receives is a fixed template filled from the case's attributions. Nothing is generated at inference, so the rendering can be regenerated, diffed and checked line by line by someone who does not trust us, and there is nothing in it to hallucinate. We pay for that in fluency and coverage, as Section 2.1 sets out.

A measurement: leakage-aware re-benchmarking of a saturated public dataset. Of the 1012 UCI Maternal Health Risk rows that survive a two-row plausibility filter, 561 are exact duplicates of an earlier row, which can place the same record in a training fold and the test fold of the same split. Scoring the identical model under the identical cross-validation with those rows retained moves accuracy from 0.641 to 0.854 and macro-recall from 0.580 to 0.859. The inflated accuracy lands inside the band the literature reports for this dataset, which is the comparison Section 3.3 sets out in full. Kapoor and Narayanan set out leakage's taxonomy and its place in the reproducibility crisis, and Rosenblatt et al. show the inflation is worst on small datasets, which is our case [12, 28]. We give it a result's billing because a reader comparing our headline number against the published band needs it to make sense of either.

Three boundaries go with those claims. **We do not claim that explanations should be audience-specific**, which was settled before us [1, 11, 13]. **We do not claim the gating mechanism.** Declining to answer under uncertainty has a long lineage in selective prediction and learning to defer, and our fixed-margin rule is a hand-set and cruder instance of it [9, 19]. **We do not claim a modelling contribution.** Fitting a classifier with SHAP attributions to this dataset is saturated [17, 18], and Section 2.5 sets out why we build on that pipeline as a

vehicle. On the audit we claim the measurement: the duplicates are the kind of thing noticed in a notebook and never written up, and we report what they cost rather than asserting we found them first.

1.2 What kind of evidence this is

This is a design and feasibility artifact, and we would rather say so here than let a reader discover it in Section 6. No mother, no community health worker and no clinician has used any of these renderings.

Our evaluation is a formative, single-evaluator heuristic critique conducted on our own artifact. It found five defects, including a driver list that could print a reading arguing against its own prediction under a heading announcing it as a flag, and all five were fixed before this paper was written. That is what the evaluation is: an instrument that improved the design. It is not evidence that any tier helps anyone, and the nearest empirical work argues against assuming so, since community health workers in India have been reported struggling with AI explanations even after simplification [24, 29].

We ask to be judged on the following. The rendering rule and the pipeline that implements it are the object; the artifact is validated by formative inspection against published usability and accessibility criteria; the leakage effect on the benchmark is quantified and reproducible in either direction from a committed table. The data here is public, the task is generic maternal-health risk, and nothing in this paper predicts an outcome for a real patient. Whether any of it helps a mother, a health worker or a clinician is a question this paper does not answer, and answering it is gated on ethics clearance rather than on effort.

Section 2 places the work against the two literatures it claims between and the three it is answerable to, Section 3 covers the dataset and the leakage measurement, and Section 4 gives the rendering rule and the three tiers. Section 5 reports the evaluation defects first, Section 6 collects what the design cannot yet support, and Section 8 gives the commands that regenerate every artifact here.

2 Related Work

Two literatures set the boundary of what we claim: stakeholder-tailored explanation and selective prediction, and our contribution is the pairing of the two rather than either mechanism. Three more set the standard we are answerable to: explainable AI for community health workers, risk communication without numeracy, and the published work on this dataset. The five subsections below take them in that order.

2.1 Stakeholder-tailored explanation

Two of our three tiers reproduce a split this literature settled before us. Arya et al. made the point in a title, *One Explanation Does Not Fit All*, and built the toolkit and taxonomy to go with it [1]. Imrie et al. show that stakeholders in healthcare impose genuinely divergent interpretability requirements, not differing degrees of one requirement [11]. Kim et al. build and compare stakeholder-tailored explanation interfaces directly, including that same clinician-versus-patient split [13]. Bello et al. propose a *three-level* pipeline from technical explanation to natural language, which is our structural twin [2]. **We therefore claim no novelty for the idea that explanations should be audience-specific, nor for tiering as an architecture.** Both predate us.

Where we diverge from Bello et al. is the mediator, and that divergence is one of our stated contributions. Theirs is a large language model. Ours is a fixed set of templates over SHAP attributions, so every string a health worker or a mother receives is determined by the case and

can be regenerated and audited. Nothing is generated at inference time and there is nothing to hallucinate. We give up fluency and coverage for a rendering an auditor can check line by line.

We name our tiers by role, and Suresh et al. argue that role is the wrong axis: a stakeholder's role determines neither the knowledge they bring nor the interpretability task they face, and they offer a typology that decouples the two [31]. The objection holds. Those names are shorthand, and what varies across the tiers is what each reader lacks, in reading modality, literacy and decision authority; each tier is designed against that deficit. In this deployment role and deficit track each other closely, since the ASHA is a specific, minimally-schooled cadre and the mother may not read at all. Szymanski et al. report that role and expertise co-vary in practice while remaining analytically separable [32], which supports role as a serviceable proxy here without rescuing it as a general axis. Where the two come apart, a deployment would have to select on the deficit.

2.2 Uncertainty communication, abstention, and selective prediction

Our gate declines the highest-severity signal when the top-two class probabilities are separated by less than a fixed margin. That is a hand-set instance of the reject option Hendrickx et al. survey, and a cruder one: their taxonomy names the case we implement, rejection under *ambiguity*, where two classes are close, as distinct from rejection under novelty [9]. Mozannar and Sontag give the learning-to-defer version, where the model is trained to route a case to a human expert rather than answer it [19]. **We claim none of that mechanism.** From Kompa et al. we take the clinical framing that licenses derating a signal instead of issuing it: in medical machine learning, abstaining on high-uncertainty cases is a safety property rather than a shortfall in coverage [15].

What we add sits one step downstream, and Bhatt et al. are the closest prior work to it [4]. They treat uncertainty as itself a form of transparency and consider explicitly how it should be *communicated to different stakeholders*. The difference is narrow: they make the *display* of uncertainty audience-aware, whereas we make the *suppression* of an assertion audience-aware. The mother does not receive the uncertainty rendered more gently, she receives a different assertion, while the clinician receives the split probabilities unmodified. Section 4 gives the rule that grades those assertions. We find no prior work that makes the abstention *rendering* a function of stakeholder vulnerability. That intersection, and only that intersection, is what we claim.

2.3 Explainable AI for community health workers in the Global South

Our second tier renders to a community health worker, and this is the literature that sets the standard it has to meet. Okolo et al.'s systematic review of explainable AI in the Global South records how little of this work reaches the people it describes: across the corpus they surveyed, deployment with actual users is close to absent [23]. That is the gap this paper sits in.

Three studies set our context. Okolo et al. interviewed community health workers in rural India about AI-enabled mobile health tools and about explanations specifically [22, 24]. Solano-Kamaiko et al. built and studied an explorable explanation interface with the same cadre [29]. Srinidhi et al. document ASHA Kirana, a deployment in which ASHAs collect data that feeds an algorithm and returns a report for a doctor [30], which is our data flow, minus the tiering and the uncertainty gate.

Two findings in that context bound what our card can be said to do. This literature does not report that simplification suffices: Okolo et al. and Solano-Kamaiko et al. both find community health workers struggling to interpret AI explanations even when those explanations have been simplified [24, 29]. The comprehensibility of our ASHA card is therefore an open empirical question, and that finding is why a user study is the necessary next step rather than an optional one. And every paper named in this subsection has users; this paper has none. We propose a

rendering rule and evaluate the artifact that implements it, which is what a design and feasibility artifact can establish, and not a claim to know better than they do what a community health worker needs.

2.4 Communicating risk without numeracy

Our third tier renders to a reader who may not read, and this literature both warrants that tier and argues against how we built it. Galesic et al. show that icon arrays let low-numeracy readers extract risk information they otherwise miss [7], and García-Retamero and Cokely's systematic review finds visual aids improve risk literacy most among the vulnerable readers our tier targets [8]. Richter et al. add the negative result that matters most for a voice channel: purely verbal communication of benefits and harms performs poorly for patients with limited health literacy [27], which is why our spoken message is paired with a visual rather than shipped alone. Peters et al. collect current best practice for numeric risk [25], and Bradley et al. test risk presentation in the maternity setting [5].

That literature recommends conveying *magnitude* transparently, through an icon array or a visible denominator. Our mother tier strips magnitude entirely and renders an ordinal band plus a single action, which read flatly is what the field advises against. We take the departure on a condition about when a magnitude is the sender's to assert, and state it in Section 4 beside the tier it governs.

2.5 Machine learning on the UCI Maternal Health Risk dataset

We build on the tree-ensemble and SHAP pipeline this dataset has standardised and use it as a vehicle, which puts what is ours downstream of it, in the rendering. Supervised classification with SHAP attributions here is saturated: Mamun et al. [18], Maisoon et al. [17], Widyawati et al. [33], Rahman et al. [26], Maheswari et al. [16] and Hossen et al. [10] all report the same broad construction: tree ensembles, resampling for class imbalance, cross-validation, and SHAP for post-hoc attribution. **There is no machine learning contribution available here and we do not claim one.**

Maisoon et al. are our closest methodological neighbour, so we separate the two explicitly [17]. They use the same dataset with the same two-row outlier removal, tree ensembles with SMOTE, stratified five-fold cross-validation, SHAP, and the same low-resource framing. Everything upstream of the explanation is substantially their pipeline. What is ours lies downstream: deduplicating before splitting and reporting what that costs (Section 3), gating the rendering on the model's own uncertainty, and rendering the gated output three ways. Their attributions and Widyawati et al.'s independently rank blood sugar and systolic blood pressure at the top [17, 33], as ours do, which corroborates the attributions rather than adding a finding of ours.

Yesmin et al. are the closest contemporaneous neighbour [34]. They pair ante-hoc fuzzy rules with post-hoc SHAP on this dataset and validate the result with fourteen clinicians in Bangladesh, 71.4% of whom preferred the hybrid explanation. Their question is which explanation format a clinician trusts; ours is whether an assertion should be issued at all, and nothing in their pipeline withholds one when the model is uncertain. Their validation also sits on the one tier our gate leaves untouched. What a health worker or a mother makes of a derated signal is not a question their study asks, and it is not one this paper answers either.

3 Data and the Model Vehicle

3.1 Dataset

We use the UCI Maternal Health Risk dataset (repository id 863), released under CC BY 4.0. It contains 1014 records collected in rural Bangladesh, each with six features (age, systolic and

Table 1: The same model and the same cross-validation, scored with exact duplicate rows dropped and retained. Mean ± standard deviation across five stratified folds.

Duplicates	n	Macro-recall	Accuracy	High-risk recall
Dropped	451	0.580 ± 0.027	0.641 ± 0.025	0.723 ± 0.054
Kept	1012	0.859 ± 0.026	0.854 ± 0.026	0.908 ± 0.031

diastolic blood pressure, blood sugar, body temperature and heart rate) and a three-level risk label: low, mid, high. It is public data and no other data is used anywhere in this work. No real patient record, de-identified or otherwise, appears in this paper or in the repository that accompanies it.

The dataset supports *generic maternal-health risk* and nothing narrower. It carries no label for any specific condition, and none of the laboratory, urinary or obstetric-history fields that would let one be inferred. We use the generic term throughout, in the manuscript, in the code and in every rendered artifact, and no tier names a condition.

The label is a risk stratification recorded alongside the readings, not a delivered outcome. Nothing in this paper should be read as a prediction of what happens to any patient, and that constraint binds the third tier hardest: a spoken message naming a diagnosis would assert something the data cannot support. What we demonstrate is an explanation design over a model fit to this public dataset.

3.2 Deduplication, and why it happens before the split

Two preprocessing steps are applied, both before any train/test split, and both chosen so that neither learns a statistic from the data.

First, two records report a heart rate of 7 beats per minute. These are physiologically impossible and are dropped, leaving 1012 rows. Second, the remaining set contains a large number of exactly repeated feature-and-label vectors: 561 of the 1012 rows are exact duplicates of an earlier row. We drop them, leaving **451 distinct records**: 233 low, 106 mid and 112 high, an imbalance of about 2.2:1 between the largest and smallest class.

Both steps are fixed rules rather than fitted transformations, which is why performing them before the split introduces no leakage: nothing about the test fold informs what is dropped. Everything that *does* learn from data, meaning feature scaling and SMOTE where a configuration used it, is fitted inside each cross-validation training fold only, through an `imblearn` pipeline. We considered the alternative, which is to retain the duplicates under group-aware cross-validation and keep more rows. We rejected it on two grounds: it adds machinery a vehicle model does not need, and deduplication makes our number directly comparable to what a reader gets from a plain `drop_duplicates`.

Deduplication does not resolve every ambiguity in the data. Among the 451 distinct records there remain **35 feature vectors that carry more than one risk label**: identical six-value readings recorded against different levels of risk. No model, however specified, can separate rows it cannot distinguish. We retain these rows rather than adjudicating between them, and return to what they imply for the achievable ceiling in Section 6.

3.3 A leakage-aware re-benchmark: what the duplicates were worth

The duplicate rows cost accuracy its meaning. A duplicated row can appear in a training fold and in the test fold of the same split, so retaining them lets a model be scored partly on records it has already memorised. We measured that effect directly. Scoring the identical model under the identical five-fold cross-validation twice, changing only whether duplicates were dropped, gives Table 1 (`results/tables/p1_duplicate_leakage.csv`).

Table 2: Six configurations under the same five-fold stratified cross-validation on the 451 deduplicated records. Mean ± standard deviation across folds. The two random-forest rows tie on the primary metric at 0.580; Section 3.4 gives the tie-break. Mid-risk recall does not exceed 0.302 in any configuration.

		Overall		Per-class recall		
Model	Resample	Macro-recall	Accuracy	Low	Mid	High
Logistic regression	none	0.553 ± 0.056	0.665 ± 0.048	0.948	0.075	0.635
Logistic regression	SMOTE	0.548 ± 0.051	0.587 ± 0.037	0.682	0.283	0.678
Random forest	none	$\mathbf{0.580} \pm 0.027$	0.641 ± 0.025	0.790	0.227	0.723
Random forest	SMOTE	$\mathbf{0.580} \pm 0.040$	0.627 ± 0.039	0.742	0.283	0.713
XGBoost	none	0.558 ± 0.020	0.621 ± 0.017	0.777	0.236	0.660
XGBoost	SMOTE	0.578 ± 0.022	0.627 ± 0.024	0.747	0.302	0.687

Retaining the duplicates moves accuracy from 0.641 to 0.854 and macro-recall from 0.580 to 0.859. The inflated *accuracy*, 0.854, is the like-for-like comparator for the published figures, which are accuracies, and it sits inside the band typically reported for this dataset; the published range extends higher still. Maisoon et al. report 83.74% and Mamun et al. 99.51% [17, 18]. Neither describes duplicate handling, both report $n = 1014$, and preprocessing on this dataset is generally given only as normalisation and the removal of inconsistencies. Yesmin et al. report 88.67% on the same $n = 1014$ and likewise describe no duplicate handling, though that figure is not a like-for-like comparison: they add a synthetic regional access score to the six clinical features [34].

Leakage is well documented: Kapoor and Narayanan set out its taxonomy and its role in the reproducibility crisis [12], Rosenblatt et al. show that leakage through repeated subjects inflates performance and that the effect is worst on small datasets, which is exactly our situation [28], and Bernett et al. give the practitioner's checklist [3]. The contribution type is established too: re-analysing an inflated published result to recover the true number is what Eltawil et al. and Young et al. do [6, 35]. **We do not claim to be the first to notice these duplicates.** We searched for prior documentation of them in this benchmark and found none, but absence of evidence is weak here: a `drop_duplicates` result is exactly the kind of thing noticed in a notebook and never written up. What we claim is the measurement: the published band on this dataset is recoverable only when identical rows are allowed to straddle the split, and after deduplication macro-recall on this vehicle is 0.580. It is reproducible in either direction from the committed table. We report deduplicated figures throughout and state the comparison here, up front, so that our lower headline number is legible as hygiene rather than as a weaker model.

3.4 The model is a vehicle

We compared logistic regression, random forest and XGBoost, each with and without in-fold SMOTE, under five-fold stratified cross-validation. Table 2 gives all six configurations, typeset from `results/tables/p1_model_metrics.csv`. The primary metric is **macro-recall**, with macro-F1, per-class recall, one-versus-rest ROC-AUC and the confusion matrix as support. Macro-recall is the right primary here because the costs are asymmetric: failing to flag a high-risk mother is the error that matters, and headline accuracy on an imbalanced three-class problem conceals that failure.

The final model is a **random forest with no resampling** (`n_estimators=300`, `random_state=42`). It ties the SMOTE-resampled random forest on macro-recall at 0.580. We broke that tie deliberately rather than by argmax: the unresampled model has tighter variance across folds (±0.027 versus ±0.040), the better high-risk recall (0.723), and no dependence on a synthetic-oversampling step. It also lets SHAP run on the six raw clinical features in their own units, which is what makes the ASHA and mother tiers expressible as templates over

attributions at all. The selection is a constant in the code, and the script still prints the argmax winner so the tie stays visible.

We take 0.580 as the operating figure for this deduplicated dataset; the rendering question exists precisely because models on data like this are uncertain. Table 2 says where that uncertainty is concentrated. Mid-risk recall is 0.227 ± 0.057 for the selected model and reaches no higher than 0.302 in any of the six configurations, against 0.790 for low risk and 0.723 for high. The mid class is the one the vehicle cannot resolve, which is what the 35 feature vectors carrying more than one risk label (Section 3.2) would predict: rows the model cannot distinguish it also cannot separate. Cases therefore land on the mid-high boundary as a structural property of this data rather than as an occasional accident, and a rendering that reaches a mother has to have an answer for them. The attributions themselves are stable and unremarkable: blood sugar is the leading global driver for every class, with systolic blood pressure second, and diastolic pressure and heart rate close to noise (`results/tables/shap_global_mean_abs.csv`). We say "blood sugar and systolic pressure" rather than "the blood-pressure features" throughout, because the data supports the first and not the second.

3.5 Maternal age

The dataset contains very low maternal ages, reflecting adolescent pregnancy in the source population: 95 of the 451 distinct records are under 18 and the minimum is 10, both written to `p1_cohort_summary.csv` by the preparation step. We retain rather than filter these rows, because excluding a group the system would encounter is itself a harm. Our designated failure case shows what that costs: a 13-year-old whose recorded body temperature of 101 °F drives a true-low record to a predicted-high output, with blood sugar and systolic pressure both arguing against the flip. The model is over-weighting a demographic feature, which is why final judgment stays with a human and why the rendering rules of Section 4 exist. All data here is public; no real patient or child record is exposed.

4 Three Tiers from One Prediction

4.1 What the alarm is, and what derating it means

By *alarm* we mean the highest-severity signal a tier is permitted to raise: the red lamp and the urgent spoken message for the mother, the HIGH header on the health worker's card. The rule this section describes withholds that signal when the model is close to a coin flip, and substitutes a middle signal that still says *needs follow-up*.

The direction of that substitution is the whole design, so we state it before anything else. **The derate is red to amber, never red to green.** A near-tie does not mean relax. It means the system is not entitled to the loudest thing it can say, and falls back to the one below it, which still sends the mother to a health worker. Nothing here removes a mother from care.

4.2 The band rule

Each rendering is driven by one band, computed once per prediction from the class probabilities and shared across all three tiers:

- GREEN if the predicted class is low risk.
- RED if the predicted class is high risk *and* the top-two probability margin is at least 0.15.
- AMBER otherwise — which covers every mid-risk prediction and every high-risk prediction that sits inside the margin.

The threshold of 0.15 is a design constant. We did not tune it, and no experiment in this paper supports that value over 0.10 or 0.20; we return to what would be needed to justify it in Section 6.

The gate is one-sided, and we state where it fires rather than leave a reader to infer it. The margin test guards the high boundary only: GREEN follows the argmax with no margin test at all, so a prediction that is low risk by a hair renders an unqualified all-clear to the mother. That is the opposite of the asymmetry Section 3.4 argues for, where missing a higher-risk mother is the error that matters. The symmetric extension, derating an ungated near-tie GREEN down to AMBER as the red-to-amber move already does, is the natural next version of this rule and is future work. This version does not implement it.

Computing the band once matters as much as the threshold does. In the first version of the renderer the health worker's card banded on its own rule and the mother's lamp banded on this one, so a single prediction could carry two identities. Section 5 describes how that was caught.

4.3 One prediction, three renderings

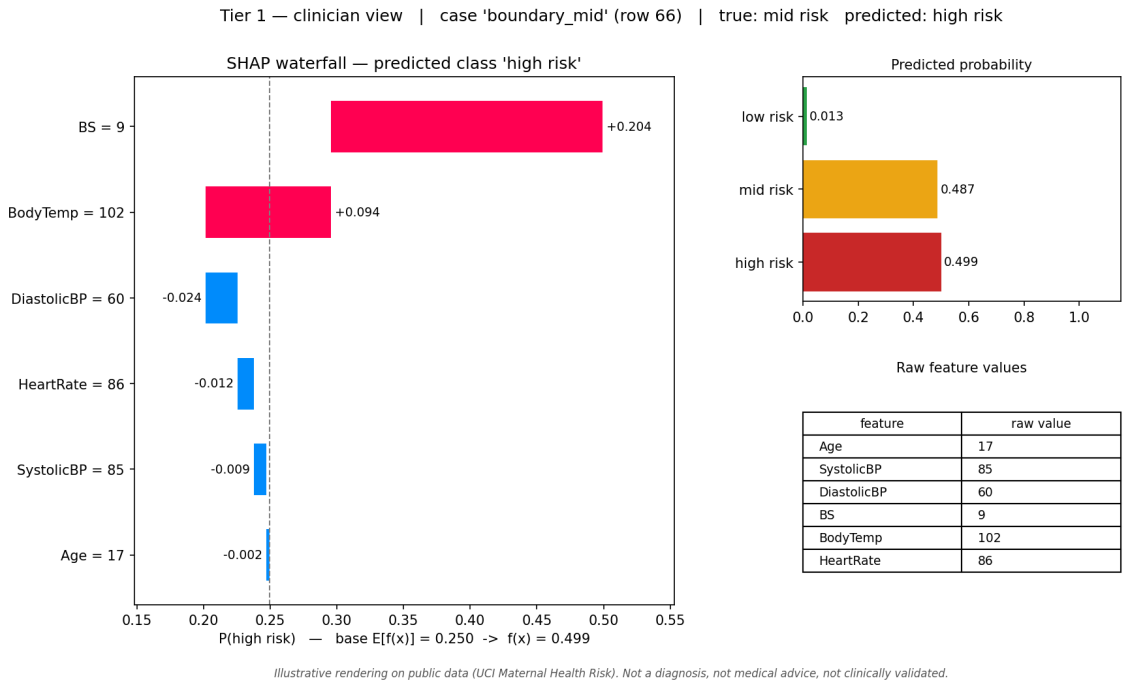
Figure 1 shows the same prediction rendered three ways. The case is row 66 of the deduplicated set, a true mid-risk record the model calls high risk, with class probabilities 0.013/0.487/0.499 across low, mid and high. The top-two margin is 0.012. Blood sugar contributes +0.204 to the predicted class and body temperature +0.094; the remaining four features contribute negatively and together amount to less than 0.05.

Clinician. The full decomposition, unmodified. Signed SHAP contributions per feature with numeric labels, all three class probabilities, and the six raw readings in clinical units. The 0.487 against 0.499 is shown as it stands. A reader who can interrogate the model is given what she needs to do so, including the fact that the call was nearly a tie.

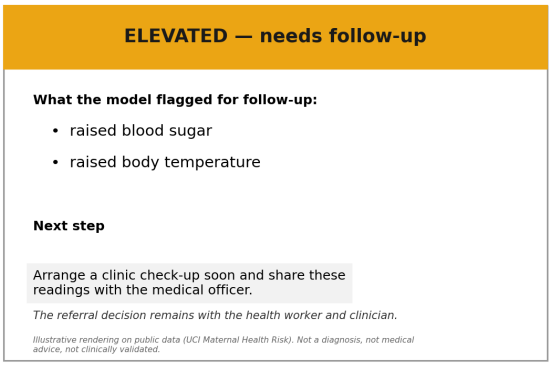
Community health worker. A card carrying the band header, the factors behind *this* prediction, and one next step. Two rules shape it. The factors are local rather than global: naming the dataset's top features on every card would print the same two words for every mother and explain nothing about any of them. And only features that push *toward* the predicted class are listed, ranked by signed contribution, because a card headed "what the model flagged" that lists a reading arguing against its own prediction misleads the person making the referral. On this case the card reads ELEVATED — NEEDS FOLLOW-UP, lists raised blood sugar and raised body temperature, and asks the worker to arrange a check-up and share the readings. Every string is a fixed template constant filled from the attributions. No language model runs at inference, which is our stated alternative to LLM-mediated tiering [2]: the card is regenerable, diffable, and has nothing to hallucinate.

Mother-to-be. A three-lamp visual and one spoken Hindi sentence. No probability, no percentage, no condition named, and no text required to use it. The tier answers *what do I do next*, not *how likely is this*. Here the amber lamp lights and the message says, in Hindi, that some findings need attention, not to panic, and to get checked soon at her ANM or nearest health centre. The Hindi strings were written and clinically reviewed by the second author, a physician and native speaker; they are neither machine-translated nor author-invented.

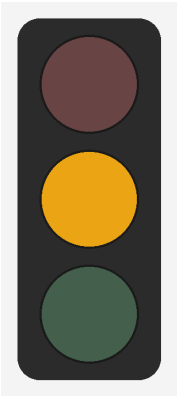
Stripping magnitude departs from the risk-communication literature of Section 2.4, which recommends conveying magnitude through an icon array or a visible denominator, and we take that departure on a stated condition. An icon array communicates a magnitude the sender is entitled to assert. At 0.487 against 0.499 the model is not so entitled, and rendering that split as roughly fifty chances in a hundred would carry a precision it does not have, to the reader least equipped to discount it. Where the magnitude is trustworthy, show it; where the gate has fired,



(a) Clinician: signed SHAP contributions, all three class probabilities, and the raw readings.



(b) Community health worker: the band header, the two factors pushing toward the prediction, and one next step.



(c) Mother-to-be: amber lit, red dark.

Figure 1: Row 66 of the deduplicated set, rendered for all three readers. The clinician panel carries the near-tie intact at 0.487 against 0.499. The card and the lamp carry the derated band, and the red they would otherwise have shown is withheld from both. Amber still tells the mother to get checked. All four cases across all three tiers are in the archived repository.

an ordinal band and one action is what the model can support. That is the uncertainty argument of Section 2.2 arriving from the health-communication side, and it remains an argument rather than a result, since whether the band is understood as intended is untested here.

4.4 Derating a false alarm

Row 66 shows the rule behaving correctly on a near-tie between mid and high. Row 382 shows what it is for. That record is true low risk, the model calls it high risk at 0.451/0.097/0.453 across low, mid and high, and the top-two margin is 0.002. A 13-year-old with a body temperature of 101 °F, the case Section 3.5 designates as the vehicle's failure mode, would have received a

red lamp and an urgent spoken message on the argmax alone. The gate withheld it and both derated channels show amber instead.

Amber on a true-low record is still a false referral. The gate made the error smaller, not correct, and it did nothing about the underlying misprediction, which the clinician tier carries unmodified here as it does on every case. What it changed is what the mother was told.

4.5 What we claim

Section 2 places the three literatures this design sits in and states what each has settled: tiering explanations by audience, withholding a prediction under uncertainty, and fitting a classifier with SHAP attributions to this dataset. We claim none of them.

What we claim is the intersection, and it is narrow. Prior work on audience-specific uncertainty makes the *display* of uncertainty audience-aware [4]; the reader with less expertise receives the same uncertainty in a gentler presentation. Here the abstention decision is taken once and then rendered as *different assertions*. The clinician receives 0.487 against 0.499. The health worker and the mother both receive amber, and the red they would otherwise have received is withheld from each. The gate draws its line at who can interrogate the model: the reader who can argue with it keeps the split, and the two who cannot lose the alarm. What separates those two is how much of the model's state each is asked to hold, not severity. The card carries the drivers and a next step; the lamp carries neither.

Both cases in this section are ones the model gets wrong. The rendering that reaches the mother is the one that survives being wrong.

5 Formative Heuristic Evaluation

We rated the rendered artifacts against WCAG 2.1 and Nielsen's usability heuristics: 16 criteria across three tiers, 48 cells, each grounded in a named detail of a file on disk. Contrast ratios are computed at run time from the palette constants rather than typed in by hand.

Both instruments are inspection methods, each with a stated procedure and a literature on what it does and does not catch. Heuristic evaluation was introduced as a way for an inspector rather than a user to find usability problems against a small set of stated principles, and the set we rate against is the refined ten [20, 21]; conformance inspection against the WCAG 2.1 success criteria is the corresponding instrument on the accessibility side [14]. Neither substitutes for users. Both were built to find defects early.

This is a single-evaluator, author-conducted, formative assessment. It is not user testing and it is not validation. Nothing in it is evidence that the tiers help anyone. What it did was find defects in our own artifact, which is what we report.

5.1 What it found

The first pass ran against the original renderer and returned four Fail ratings, three of them on the health worker's card, arising from five distinct defects. All five were fixed and the tiers re-rendered; the ratings below and in Table 3 describe the corrected artifacts.

The driver list was sign-blind. Features were ranked by absolute SHAP magnitude with no sign filter, so a reading that argued *against* the prediction could be printed under a heading announcing it as a flag. On the false-positive case the card listed low blood sugar, whose contribution to the predicted class was -0.085. The routine card was worse: it listed the two features that were the model's reasons for saying *low* under that same heading, above a next step telling the reader to arrange a check-up. This is the defect that could have put a wrong cue in front of the person making a referral, and no model metric would have shown it.

Table 3: All 48 cells: 16 criteria rated against each tier, on the corrected artifacts. • pass, ○ partial, × fail, – deferred. The per-cell rationale for every rating is in `results/tables/heuristic_matrix.csv`. Read by row the criteria are covered; read by column each tier fails where a single universal rendering would have to compromise.

Criterion	Clinician	ASHA	Mother
WCAG 2.1			
1.1.1 Text alternatives for non-text content	○	•	○
1.4.1 Colour not the only cue	•	•	○
1.4.3 Contrast (minimum)	•	•	•
1.4.5 Images of text	×	○	•
2.1 / 2.4 Keyboard, focus order, navigation	–	–	–
3.1.1 / 3.1.2 Language of page and of parts	•	○	•
3.1.5 Reading level / plain language	○	○	•
3.2.4 Consistent identification	•	•	•
4.1.2 Name, role, value	–	–	–
Nielsen's heuristics			
1. Visibility of system status	•	•	•
2. Match between system and the real world	•	•	•
4. Consistency and standards	•	•	•
5. Error prevention	•	•	•
6. Recognition rather than recall	○	•	○
8. Aesthetic and minimalist design	○	•	•
10. Help and documentation	○	•	○

The two tiers banded independently, so the confident high-risk case rendered an amber card beside a red lamp — one prediction with two identities. The band is now computed once and passed to both.

Header contrast was assumed rather than measured. White header text was hard-coded; on the amber header, which is the state for two of the four rendered cases, it measured 2.12:1. Ink is now chosen per band by measured contrast, worst case 5.14:1, and a self-check asserts it.

Direction words had no deadband, so a blood sugar of 7.7 against a median of 7.5 was worded as "raised". The word now attaches only outside 0.25 IQR of the median.

The mother's transcript carried no provenance, while both other tiers printed the disclaimer. It now records case, band and disclaimer under an explicit written-not-spoken rule; the spoken line is unchanged, because appending provenance to every message would bury the one action the tier exists to deliver.

5.2 What it could not settle

Table 3 gives the full matrix. Across the 48 cells the distribution after those fixes is 29 pass, 12 partial, 1 fail and 6 deferred. The count is a description of coverage and not a score, and the five defects above carry more weight. The single failure is the clinician tier shipping as an image of text. The six deferred cells are all keyboard operability and assistive-technology semantics, which a static PNG cannot exercise and a deployed application would.

Read by column, each tier fails where a single universal rendering would have to compromise: the clinician view needs chart and English literacy, the mother's view needs hearing, and neither gap closes inside its own tier without destroying what that tier is for. Read by row, the criteria are covered. The architecture is what conforms; no single output does.

None of this tells us whether a health worker or a mother understands what she is handed. That question needs users, and the nearest prior work is not encouraging about assuming the answer: studies with community health workers in India report difficulty interpreting AI explanations even after they have been simplified [24, 29]. Studies with mothers, health workers

and clinicians are the next step, and they are gated on ethics clearance.

6 Limitations

6.1 Evaluation without users

No mother, health worker or clinician has seen any of these renderings. That is the limitation the rest depend on, and it bounds what the paper is allowed to conclude: we have a design and a formative critique of our own artifact, not evidence that the design works.

Four claims sit downstream of it. Whether a health worker reads the card as intended is assumed rather than shown, and the nearest evidence argues against assuming it, since community health workers in India have been reported struggling with AI explanations even after simplification [24, 29]. Whether withholding magnitude from the mother helps or harms her is an argument from the model's uncertainty, and Section 2.4 makes it, but the risk-communication literature it departs from is empirical and our departure is not. The 0.15 margin is a design constant with no experiment behind it, as Section 4 states. And role remains a proxy: we design against what each reader lacks in modality, literacy and decision authority, but we recruit nobody to check that the proxy holds, which is the substance of the objection Suresh et al. raise against role-based tiering [31].

All four resolve the same way. Studies with mothers, community health workers and clinicians, gated on ethics clearance, which this preprint does not have and does not claim.

6.2 What one public dataset can support

The work uses the UCI Maternal Health Risk dataset and nothing else. It is public, it carries generic three-level risk labels, and it names no clinical condition, so neither do we. Nothing here is clinically validated and nothing predicts an outcome for a real patient.

Two properties bound the vehicle. Macro-recall is 0.580 after deduplication, which we report as the honest figure rather than the inflated one (Section 3.3); and 35 of the 451 distinct records are feature vectors carrying more than one risk label, an ambiguity no model can resolve from these six features. That is a ceiling on the data, not a shortfall in the classifier.

One assumption inside the gate deserves naming. The margin rule thresholds a difference between random-forest class probabilities, and those are vote fractions rather than calibrated posteriors. We treat 0.15 as a design constant on an ordering, not as a statement about probability. Calibrating the model, and then asking whether the threshold should move, is future work.

The gate also has only one side. It tests the margin at the high boundary and not at the low one (Section 4), so a near-tie that resolves to low risk renders an unqualified GREEN to the mother and no derate is available for it. Extending the rule symmetrically is future work; this version does not do it.

6.3 Who the renderings still exclude

Each tier assumes something about its reader.

The mother's visual carries no rendered text, so a deaf or hard-of-hearing mother gets nothing from it; the spoken sentence is her only channel and she cannot hear it. A rendered caption or an SMS companion would close this, and it reopens the literacy assumption the tier exists to avoid, so it needs user input rather than a unilateral design call. Colour is the lamp's primary cue. Position and the lit-versus-unlit luminance step are real redundancies and the spoken line is a second channel, but a reader who is both colour-blind and deaf is left with position alone, and position assumes the traffic-light convention. The clinician view needs chart literacy and English. The health worker's card is English-only, which is a scoped exposition choice rather

than a claim about the architecture, since every string is a fixed template constant and localising it is substitution deferred to deployment. All three tiers ship as rasterised images of text.

6.4 The voice is a stand-in

Audio is synthesised with a general-purpose engine reached over the network. It is not tuned for Indian-language health speech, it handles acronyms unpredictably, and it needs connectivity that the setting this design describes may not have. Deployment-grade Indian-language synthesis (AI4Bharat, Bhashini) is the stated path. What we treat as the artifact is the Hindi source string, not the audio file.

6.5 Evaluated on a workstation

This is a design and feasibility artifact evaluated on a workstation. We make no field claim, no claim about performance under real clinic conditions, and no claim that the renderings have been carried to anyone by any delivery channel.

7 Conclusion

We built a rendering pipeline that takes one maternal-health risk prediction and issues three different assertions from it, one per reader, withholding the highest-severity signal from the two channels that cannot interrogate the model whenever the top-two class probabilities fall inside a fixed margin. Every string in it is a template filled from the case's SHAP attributions and no language model runs at inference, so any rendering here can be regenerated and checked line by line. We also re-benchmarked the dataset underneath it: 561 of the 1012 usable rows are exact duplicates, retaining them across the split moves macro-recall from 0.580 to 0.859, and 0.580 is the figure this vehicle earns once they are dropped.

Row 382 is the case that shows what the rule is for. The model calls a true-low record high risk on a top-two margin of 0.002, and the mother receives amber where the argmax alone would have sent red. That is still a false referral. The gate made the error smaller, not correct. Whether any of this helps a mother, a health worker or a clinician is untested here and gated on the ethics clearance Section 6 describes.

8 Reproducibility

Every figure, table and rendered artifact in this paper regenerates from five commands.

`python -m src.data` Fetches the dataset once from the UCI repository, caches it to `data/raw/`, and reports row count, duplicate count and missing values without modifying anything. Every later run reads the cache, so the rest of the pipeline needs no network.

`python -m src.model` Runs five-fold stratified cross-validation over three classifiers with and without SMOTE, fitting scaling and resampling inside each training fold. Writes `p1_model_metrics.csv`, `p1_duplicate_leakage.csv` and `p1_cohort_summary.csv`, which carries every cohort count this paper quotes. Saves the selected model.

`python -m src.explain` Computes global mean absolute SHAP per feature per class and local SHAP for the four criterion-selected cases. Writes three CSVs and five figures.

`python -m src.render` Renders the three tiers. It reads the saved SHAP tables rather than the model, so the renderer and the explainer cannot drift apart. `--case all` does all four cases.

`python -m src.evaluate` Rates the rendered artifacts across the 48 cells and writes `heuristic_matrix.csv` and `heuristic_evaluation.md`. Contrast ratios are computed from the renderer's own palette constants at run time.

8.1 Determinism

One seed, fixed at 42 in `src/config.py`, is applied to Python's hash randomisation, the standard library generator and NumPy by a `set_seeds()` call at the top of each script that touches randomness. The loader contains no random component and needs none. Dependencies are pinned to exact versions and require Python 3.12, since the NumPy and scikit-learn releases named in `requirements.txt` predate the 3.13 and 3.14 wheels and will not build on a newer interpreter. The dataset is cached after first download rather than re-fetched. Re-running the pipeline reproduces every PNG, CSV and text output byte for byte.

8.2 The exception

Speech synthesis is not ours to seed. The Hindi audio is produced by a remote service, and re-running the renderer on unchanged input returns files that are sometimes byte-identical and sometimes not. We state that as it is rather than rounding it to either extreme. The determinism guarantee above covers our pipeline and stops at that endpoint.

What is deterministic, and what we therefore treat as the artifact of record, is the Hindi source string. It lives in `results/tables/tier_mother_<case>_hi.txt`, it is committed, and it reproduces exactly. The MP3 is a rendering of that string.

One operational consequence is worth recording, because we were caught by it. Re-rendering over an existing audio file can leave the previous file in place while still reporting success and updating its timestamp. A cold run must empty `results/audio/` before re-rendering, or it can certify audio it did not regenerate.

8.3 Artifacts

The dataset is public and CC BY 4.0. Code, all rendered artifacts for four cases across three tiers, the cross-validation and SHAP tables, and the evaluation matrix are archived at `https://doi.org/10.5281/zenodo.22252076`, which is the concept DOI and resolves to the latest release; development history is at `https://github.com/stiFFLer-codes/three-voices`. The paper cites the committed tables directly, so any number in it can be checked against the file that produced it. The two-row leakage comparison in Table 1 is one of those files rather than a separate analysis: `p1_duplicate_leakage.csv` is written by the same command that writes the main metrics table, from the same cross-validation, so the comparison re-runs whenever the model does.

Declarations

Funding

No funding was received for conducting this study.

Competing Interests

The authors have no relevant financial or non-financial interests to disclose.

Ethics Approval

This study involved no human participants and no animals. It is a secondary analysis of the UCI Maternal Health Risk dataset, which is public, released under CC BY 4.0, and contains no identifiable personal data. Ethics approval was therefore not required and none was sought. The studies with mothers, community health workers and clinicians that this paper names as its next step are gated on ethics clearance that has not been obtained and is not claimed anywhere in this work.

Consent to Participate and Consent to Publish

Not applicable. No human participants were recruited and no individual person's data is reported.

Data and Code Availability

The dataset is the UCI Maternal Health Risk dataset (repository id 863), public and released under CC BY 4.0, at `https://doi.org/10.24432/C5DP5D`. No other data is used and no real patient record appears in this work.

All code, the rendered artifacts for all four cases across all three tiers, the cross-validation and SHAP tables, and the evaluation matrix are archived at `https://doi.org/10.5281/zenodo.22252076`. That is the concept DOI and resolves to the latest release; development history is at `https://github.com/stiFFLer-codes/three-voices`. Every figure and table in this paper regenerates from the five commands listed in Section 8.

Author Contributions

Maitreya Sapariya — Conceptualization, Methodology, Software, Formal analysis, Writing – original draft.

Aditi Patil — Clinical domain expertise, Hindi authoring and review, ASHA-tier design input, Validation.

Use of Generative AI in the Preparation of this Manuscript

The authors used an AI assistant (Claude, Anthropic) to draft and revise the text of this manuscript, working under author direction. The research design, the analyses, every reported number, the selection and verification of every citation, and the final wording were determined, checked and approved by the authors, who take full responsibility for the content. This is a statement about how the paper was written and not about the system it describes: the rendering pipeline of Section 4 runs fixed templates over SHAP attributions and invokes no language model at inference, which is one of the contributions claimed here.

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